CRISIS PLAYBOOK · NO. 06

- T+00:00 Incident declared
- T+00:02 Alert sent · push, SMS, voice
- T+01:30 Escalated · no response
- T+04:10 Team acknowledged



HEALTHCARE

Major incident, minute one.

Ransomware on the laboratory system, a mass casualty surge at night, a cooling or power failure in a heatwave. How hospitals declare, call staff in, switch to downtime procedures and keep patients safe.

T+00:00 · T+00:05 · T+00:15 · T+01:00 · T+04:00

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FOREWORD

In healthcare, an IT incident becomes a patient safety incident within the hour.

In June 2024, in the UK, a ransomware attack on Synnovis, the pathology provider for major hospitals in south east London, stopped normal blood testing. NHS England later reported that 10,152 acute outpatient appointments and 1,710 elective procedures had been postponed at the two most affected hospital trusts.[1] A year later one of the hospitals confirmed that a long wait for a blood test result, caused by the attack, was one of the factors in a patient's death.[2]

It is not only a UK story, and not only a cyber story. In May 2024, in the US, a cyber attack on Ascension, which operates 140 hospitals in 19 states, forced several hospitals to divert ambulances and moved staff onto paper records.[3] In June 2026 an English hospital declared a critical incident when its cooling systems failed in a heatwave.[4] Almost two years after the Synnovis attack, one UK provider was still working in business continuity for pathology.[5]

This playbook is for the people who carry the bleep when that happens, in hospitals and health systems in the UK, the US, Europe and the Middle East: emergency managers and business continuity leads, administrators and executives on call, clinical, nursing and digital leaders, and communications teams. It covers the first hour, the downtime that follows and the plan to be ready before the next one. The companion intelligence report sets out the evidence in full.

The Crises Control team
Crises Control

EXECUTIVE SUMMARY

Declare early, reach the right people, keep care going without the systems.

01 · THE OPERATIONAL CHALLENGE

- Clinical care runs on systems, suppliers and utilities that the hospital does not fully control.
- Plans exist, but the first hour often depends on who happens to be on call and who answers.
- Clinical teams, management, digital teams and external partners work from different information when it matters most.

02 · INCIDENTS AND RISKS TO PREPARE FOR

- A cyber attack or IT failure that takes pathology, the patient record or prescribing offline, at the hospital or a supplier.
- A mass casualty incident or sustained surge that needs staff called in out of hours.
- Loss of power, water, cooling or medical gases to clinical areas.
- A long downtime: weeks on paper, results to back-enter and staff to rest.

03 · KEY RECOMMENDATIONS

- Write declaration triggers and give named roles the authority to declare without waiting for certainty.
- Call staff in by role on several channels, and track who has acknowledged.
- Print, practise and time downtime procedures in every department, including transfusion.
- Put suppliers, the ambulance service and your regional health authority on your incident lists.
- Rehearse all three scenarios each year, and close every action from the review.

04 · WHAT EFFECTIVE COORDINATION LOOKS LIKE

- A declaration in agreed words within minutes, reaching wards and partners at the same time.
- One incident log and one actions board for clinical, management, digital teams and partners.
- Downtime running as owned tasks, with the time to the first paper result measured.
- Updates to staff and patients at fixed times until stand down, then a debrief within days.

05 · HOW CRISES CONTROL SUPPORTS THE RESPONSE

Most organisations can send alerts. Few can manage incidents end-to-end.

Crises Control is an Operational Incident Coordination Platform. Ping calls staff in by role by push, SMS, voice and email, and no reply escalates to a deputy under the rules you configure.

Incident Manager and Task Manager give one log and owned downtime tasks; Audit & Reporting keeps the timestamped record for your review and your regulator.

INTRODUCTION

Three things decide how the first hour goes.

Almost every hospital can send an alert. Far fewer can run an incident from first report to final review: declare it, mobilise the right people, keep care safe on paper and show what was done. The ones that cope best have rehearsed three simple things until they are habits.

Declare early, in agreed words

Define incident levels and triggers in advance, name who can declare, and use the same words with staff and partners. Where your framework sets the wording, use it.

Know who is coming in

Reach on-call staff, ward leaders and off-duty teams in minutes, and know who has acknowledged, who is on the way and who has not answered.

Make downtime safe

Paper requests, manual result reporting, safe blood and clear handovers, practised before the systems fail, so patient care does not depend on the network or the plant room.

The network can go down. Patient care cannot.

A cyber attack, a mass casualty incident or a failed power or cooling plant all end in the same place: clinicians working without the systems they rely on, and patients who need to be told what is happening.

PART 01

Declare and decide.

Agree the words, the thresholds and who can say them, before the day you need them.

01 Three incident levels, defined in advance

02 Why declaring early matters

03 Emergency preparedness rules by region

04 Reporting a cyber or data incident, by region

DEFINITIONS

Three incident levels, defined before you need them

Names vary by country and framework. What matters is that everyone, from the ward to the board, means the same thing. Illustrative definitions: use your own framework's wording where one exists.

LEVEL	WHAT IT MEANS	TYPICAL TRIGGERS
Business continuity incident	Disruption that takes a service below an acceptable level and needs special arrangements until it recovers.	One system or supplier down with a working fallback; planned downtime overrunning; a staffing gap in one area.
Critical incident	Loss of the ability to deliver critical services, or a risk of harm to patients or staff, needing support from beyond the department or site.	Pathology, the patient record or prescribing down across the hospital; suspected ransomware; power, water, cooling or medical gases lost to clinical areas.
Major incident	An event with serious consequences that needs special arrangements by one or more emergency responder agencies, often multi-agency.	A mass casualty incident; evacuation of a building; an incident affecting several hospitals or the wider community.
Declaring	One agreed format: who declares, which level, at what time. Standard messages for standby, declared and stand down.	**United Kingdom:** NHS England's EPRR Framework defines all three levels and the wording “Critical Incident declared by (organisation)”.[6]

DECLARE AND DECIDE

Why the first decision is the hardest, and the most important

> **Declaring is how you get the right people moving. Waiting for certainty puts you two hours behind the wards.**

Nobody wants to declare too soon. A critical incident is a public signal, it pulls in partners and it can feel like an admission. So the first hour often goes on fixing the problem quietly, while wards improvise and the executive on call hears about it second hand.

Declaring is how help arrives. **United Kingdom:** NHS England's framework describes a critical incident as principally an internal escalation response to disruption that will affect the ability to deliver safe patient care.[6] **United States:** many hospitals organise their response with the Hospital Incident Command System (HICS), an incident command framework with job action sheets for each role.[7] The labels differ; the purpose is the same.

Declaring is also how the right people start moving. An administrator on call who declares a business continuity incident at 08:15 can start the downtime procedures, call in the laboratory and digital leads and warn the emergency department. One who waits for certainty at 10:00 is two hours behind the wards.

So set the triggers in advance. Loss of a clinical system that supports results, prescribing or patient flow for more than a set time. Any suspected cyber attack on a clinical system. Loss of power, water, cooling or medical gases to a clinical area. A surge that takes the hospital to its highest escalation level. Write them down, give named roles the authority to declare and tell them they will never be criticised for declaring early and standing down.

Use the agreed words, and share facts in a structured form such as METHANE or SBAR, which the NHS framework sets out with an example for a cyber incident.[6] Then tell your own staff, in the same words, at the same time. A declaration that reaches the regional health authority before it reaches the wards has gone the wrong way round.

REGIONAL NOTES

Emergency preparedness: what applies where

Hospitals everywhere are expected to plan, train and exercise for emergencies. The detail, and who checks it, differs by country.

REGIONAL NOTE · UNITED KINGDOM

England: NHS-funded organisations work to NHS England's EPRR Framework and assess themselves each year against the EPRR Core Standards; the NHS Standard Contract requires providers to follow EPRR guidance.[6] Under the Civil Contingencies Act 2004, NHS England, integrated care boards and foundation trusts providing A&E or ambulance services are Category 1 responders.[8]

IN PRACTICE Map your triggers to the EPRR definitions and keep the evidence for the annual assurance return. Scotland, Wales and Northern Ireland have their own arrangements.

REGIONAL NOTE · UNITED STATES

Hospitals in Medicare and Medicaid must meet the CMS emergency preparedness rule, 42 CFR 482.15: an all-hazards risk assessment and plan, policies, a communication plan, and training and testing, reviewed at least every 2 years, with exercises at least twice a year.[9] The Joint Commission's revised Emergency Management standards apply to accredited hospitals from July 2022.[10]

IN PRACTICE The rule expects drills, exercises and real events to be analysed and documented, and the plan revised. Many hospitals use HICS to run the response.[7,9]

REGIONAL NOTE · EUROPEAN UNION

Hospital preparedness is set nationally, and systems vary. The CER Directive (EU) 2022/2557 covers health among its sectors: Member States had to transpose it by 17 October 2024 and identify critical entities by 17 July 2026. Critical entities must take resilience measures and notify incidents.[11]

IN PRACTICE Ask your national authority whether your hospital is, or serves, a designated critical entity, and align your plan with national emergency arrangements.

REGIONAL NOTE · MIDDLE EAST (UAE AND SAUDI ARABIA)

Dubai: the DHA framework for healthcare emergencies, effective 14 January 2025, requires licensed facilities to run an annual hazard vulnerability analysis, keep a 24/7 contact for DHA, give hospitals an emergency operations centre and hold at least one exercise a year.[12] **Abu Dhabi** launched a healthcare business continuity standard in May 2025.[13]

IN PRACTICE **Saudi Arabia:** CBAHI accreditation is linked to hospital licence renewal.[14] JCI surveys use its 8th edition hospital standards from 1 January 2025.[15]

A summary for orientation, not legal advice. Requirements depend on sector, size, licence and jurisdiction, and change over time. Confirm the current text with your legal and compliance teams.

REGIONAL NOTES

Reporting a cyber or data incident: the clocks differ

A cyber attack on a clinical system is an operational incident and, often, a reportable one. Know which clock starts, who files and to whom, before the day.

REGIONAL NOTE · UNITED KINGDOM

England: NHS trusts, foundation trusts, integrated care boards and certain independent providers are operators of essential services under the NIS Regulations 2018. They report network and information system incidents through the Data Security and Protection Toolkit within 72 hours of becoming aware.[16]

IN PRACTICE Name who files the report and when the 72-hour clock starts. Personal data breaches can also need a separate report to the data protection regulator.

REGIONAL NOTE · EUROPEAN UNION

Under NIS2, health is a sector of high criticality. Entities in scope send an early warning within 24 hours of becoming aware of a significant incident, an incident notification within 72 hours and a final report no later than one month later. Member States had to transpose NIS2 by 17 October 2024.[18]

IN PRACTICE The detail is in national law. Check whether your hospital is an essential or important entity, and who your CSIRT or competent authority is.

REGIONAL NOTE · UNITED STATES

Under the HIPAA Breach Notification Rule, covered entities notify affected individuals without unreasonable delay and no later than 60 calendar days after discovering a breach of unsecured health information; breaches affecting 500 or more people are reported to HHS at the same time.[17]

IN PRACTICE A ransomware attack can be both a downtime and a breach. Bring privacy, legal and digital leads into the incident team in the first hour.

REGIONAL NOTE · MIDDLE EAST (UAE)

Requirements are set nationally and, in the UAE, by each health regulator. **Dubai:** the DHA framework requires facilities to name a focal point, preferably from incident command, and keep a phone number and email that reach them 24/7 during emergencies.[12]

IN PRACTICE Confirm with your health regulator and national cyber security authority which incidents you must report, to whom and how fast. Do not assume another region's clocks apply.

A summary for orientation, not legal advice. Requirements depend on sector, size, licence and jurisdiction, and change over time. Confirm the current text with your legal and compliance teams.

PART 02

Incident scenarios.

Three scenarios every hospital should plan for: the systems go down, the patients arrive, the building fails.

01 A severity ladder for the first hour

02 Three scenarios at a glance

03 Cyber or IT downtime: the first hour, team by team

04 Cyber, mass casualty, utility failure: where each fails

05 Calling staff in and tracking outstanding actions

SEVERITY

A hospital severity ladder for the first hour

An internal scale for alerting and staffing. It sits alongside your formal declarations, not instead of them. If in doubt, grade up and step down later.

SEV 1

Major or critical, hospital wide

TRIGGERS

Loss of pathology, the patient record or prescribing across sites; suspected ransomware; mass casualty incident declared by you or the ambulance service; loss of power, water or medical gases to clinical areas.

WHO IS ALERTED, HOW

Executive on call, incident commander, all clinical leads, digital, estates, comms; regional health authority told at once.

`Push, SMS and voice · ack in 5 min`

SEV 2

Critical in one service or site

TRIGGERS

One site's emergency department, theatres or diagnostics unable to work safely; a critical system down beyond the agreed time; surge at the highest level with patient safety risk.

WHO IS ALERTED, HOW

Administrator and manager on call, service and site leads, digital, comms.

`Push and SMS · ack in 10 min`

SEV 3

Business continuity incident

TRIGGERS

Planned downtime overrunning; a single system or equipment failure with a working fallback; a staffing gap in one area; supplier outage.

WHO IS ALERTED, HOW

Manager on call, service leads, emergency management lead.

`Push and email · ack in 30 min`

SEV 4

Watch and prepare

TRIGGERS

Supplier warning, heat or cold alert, cyber advisory affecting your systems, rising pressure, a major event in your area.

WHO IS ALERTED, HOW

Emergency management lead, digital, estates, site team.

`Email and app · no ack needed`

SCENARIOS

Three scenarios at a glance

Different causes, the same first hour: declare, mobilise, switch to downtime, inform. Illustrative; adapt the triggers and owners to your hospital.

SCENARIO	TYPICAL TRIGGERS	FIRST DECISIONS AND WHO MAKES THEM	DOWNTIME AND RECOVERY
1 · Cyber attack or clinical IT downtime	Laboratory, record, imaging or prescribing systems fail; ransom note; a supplier reports an attack.	Incident commander grades and declares; digital lead isolates systems; transfusion lead sets the O type rule.	Paper requests and phoned results; reporting clocks start; restore only when declared safe; results back-entered.
2 · Mass casualty or surge	Ambulance service alert of a major incident; sudden arrivals; sustained demand at the highest escalation level.	Emergency department lead and executive on call declare; incident commander opens the command centre and stands down planned care.	Role-based call-in; beds and theatres released; blood stock checked; staff rested and rotated.
3 · Utility or building failure	Loss of power, water, cooling or medical gases; generator or chiller fault; flood or fire in a plant room.	Estates lead advises; executive on call declares and decides whether to move patients or lists.	Generator, gas and temperature checks at set times; affected services moved; servers and devices checked before restart.

THE FIRST HOUR

Scenario 1, Monday 08:10: the lab goes dark

An illustrative acute hospital. Treat it as a cyber attack until proved otherwise.

CLOCK	ACTION	OWNER
T+00:00	**Laboratory staff report the system is down** Lab manager calls the site manager and the digital service desk. No results are going out.	LAB MANAGER
T+00:05	**Manager on call grades it SEV 1** Business continuity incident declared. Executive on call and incident commander alerted. Digital isolates systems.	MANAGER ON CALL
T+00:10	**Call-in alert sent** Incident team, laboratory, transfusion, emergency department, theatres, critical care, comms. Reply with arrival time; silence escalates to a deputy at 10 minutes.	SITE MANAGER
T+00:15	**Downtime procedures start** Paper requests and phoned urgent results; transfusion to its downtime plan. Clinical leads review today's planned care.	LABORATORY LEAD
T+00:20	**Reporting clocks start** Digital and privacy leads note the time of awareness and start the cyber and data reporting routes that apply to you (see Part 01).	CHIEF INFORMATION OFFICER
T+00:30	**Critical incident declared** Command centre opened. Regional health authority told; testing mutual aid requested from a partner laboratory.	EXECUTIVE ON CALL
T+01:00	**Messages out, first meeting held** Holding lines to staff, today's patients and the emergency department. Facts in METHANE or SBAR form; decisions logged; open actions reviewed at each meeting.	INCIDENT COMMANDER

ACROSS THE TEAMS

The same hour, team by team: where it goes wrong

Clinical teams, incident command, digital, partners and comms in the laboratory outage. Red cards are the points where responses commonly break down; diamonds are decisions.

	T+0	T+10 min	T+30 min	T+1 hour	T+4 hours	Day 2 on
Clinical teams	Lab reports results not flowing	! Wards keep sending electronic requests	Paper requests; urgent results phoned	◆ Clinical leads triage today's planned care	Results logged on paper for back-entry	! Same staff on paper for days, no rota change
Site management and incident command	◆ Manager on call grades SEV 1	Call-in sent; acknowledgements tracked by role	◆ Executive on call declares critical incident	One log; actions owned and timed	Fixed meeting and situation report times	Recovery cell and second shift planned
Digital and IT	! Service desk logs a fault, not an incident	Systems isolated; reporting clocks noted	Out-of-band channel confirmed for staff		◆ Restore only when declared safe	Back-entry checked against paper logs
Partners: health authority, ambulance, supplier	! Supplier's incident call leaves out the hospital		Health authority told; mutual aid asked	Ambulance service warned of possible diverts	Mutual aid turnaround tracked by batch	
Comms		! Staff hear first on personal messaging apps	Holding line to staff and switchboard	Patients booked today sent SMS	Updates at fixed times until stand down	

Action · Decision · Where it goes wrong · Monitored

> Illustrative. Most failures sit at hand-offs between teams, not inside them. Give every lane the same incident, the same log and the same update times, and agree in advance how staff will talk if email and chat cannot be trusted.

IN PRACTICE

Three scenarios, three first hours

SCENARIO 1 · ACUTE HOSPITAL, THREE SITES

Cyber attack at a laboratory partner

WHERE IT FAILS

The laboratory is run by a partner; nobody at the hospital was on its call list, so wards heard first.

WHAT HAPPENS

A joint call-in list and shared incident log with the partner; downtime forms printed on every ward.

WHO DECIDES

The executive on call declares; the transfusion lead authorises O type use, agreed with the blood bank.

HOW IT IS MONITORED

Time from first report to first paper result, measured in a joint exercise and again on the day.

SCENARIO 2 · REGIONAL TRAUMA CENTRE

Mass casualty incident at night

WHERE IT FAILS

The ambulance alert reaches the emergency department, but calling in theatres, critical care and the blood bank relies on a phone tree.

WHAT HAPPENS

Role-based call-in: coming, later, not available. Planned lists stood down; beds and theatres updated at set times.

WHO DECIDES

The emergency department lead and executive on call declare; the incident commander opens the command centre.

HOW IT IS MONITORED

Acknowledgements by role against the casualty estimate, staff arrival times and beds released.

SCENARIO 3 · HOSPITAL WITH A SHARED PLANT ROOM

Cooling and power failure in a heatwave

WHERE IT FAILS

Theatres, scanners and the server room shared one cooling plant, as one English hospital found in June 2026.[4]

WHAT HAPPENS

Lists moved to a partner site; generator, fuel and medical gas checks run as tasks.

WHO DECIDES

The estates lead advises; the executive on call declares and decides whether to move lists and patients.

HOW IT IS MONITORED

Temperatures, generator and gas supply checked at set times against triggers.

CALL-IN

Five rules for calling staff in

Most hospitals have a cascade. Few know, in real time, who has answered. It matters most in a mass casualty incident at 03:00.

01 Alert by role, not by name

Lists are built around roles and rotas: the on-call transfusion scientist, the theatre coordinator, the nurse in charge. People change; roles do not.

02 Ask for an answer, not a read receipt

Every call-in asks a question: available now, available later, not available. Silence after the set time escalates to a deputy, under rules agreed in advance.

03 Use more than one channel

Push, SMS and voice. If the incident is a cyber attack, assume hospital email and chat may be down or untrusted.

04 Protect people off duty

Ask for what you need and no more. Track hours so the people who come in at 08:30 are not still there at midnight. Plan the second shift in the first hour.

05 Stand down in the same way

Release staff with a message as clear as the one that called them in, and tell them when the hot debrief will be.

REMEMBER

If you cannot see who has acknowledged within ten minutes, you do not yet know whether you have a response team.

OUTSTANDING ACTIONS

Scenario 1, day one, 16:00: the actions board

Illustrative board for the laboratory outage, reviewed at every incident meeting. Overdue and blocked items go straight to the executive on call.

ACTION	OWNER	DUE	STATUS	HOW IT IS CHECKED
Paper request forms on every ward	Laboratory lead	Day 1 10:00	DONE	Runner signs off each ward
O type authorisation rule confirmed	Transfusion lead	Day 1 09:00	DONE	Decision and authoriser in the log
Patients booked today contacted	Outpatients manager	Day 1 11:00	DONE	SMS delivery and call log
Mutual aid testing route agreed	Incident commander	Day 1 12:00	IN PROGRESS	Samples sent against results back
Cyber and data incident reports filed	Chief information officer	Per your clocks	IN PROGRESS	Reference numbers in the log
Check-ins for community and lone staff	Community manager	Each shift	OVERDUE	Missed check-ins escalated, not waited on
Second shift and welfare plan	Incident commander	Day 1 14:00	OVERDUE	Hours worked against the rota
Results back-entry plan	Digital lead	Day 3	BLOCKED	Waiting for assurance systems are clean
Situation report to the health authority	Incident commander	Daily 16:00	OPEN	Sent from the log, not from memory

> Every action has one owner, a due time and a way to check it is done. If an action has no check, nobody will know it has stalled. The incident commander owns the board and reviews it at every meeting and handover; closed actions stay visible for the debrief.

PART 03

Keep patients safe.

Downtime, partners, patients and staff, from the first hour to the last debrief.

01 Downtime that works, department by department

02 A downtime checklist and a results log

03 Partners and long incidents

04 Your plan: first week, month and quarter

KEEP PATIENTS SAFE

Downtime procedures: the plan you hope never to use

> **Can a ward request an urgent blood test, get the result and act on it safely with no computers at all?**

Most clinical areas have a downtime folder. The question is whether anyone has opened it this year. The test is simple: can a ward request an urgent blood test, receive the result and act on it safely with no computers at all? If the answer depends on one person who knows how, it is not a procedure.

Downtime can last far longer than plans assume. **United Kingdom:** after the Synnovis attack, one provider estimated in early 2026 that entering 161,560 pathology reports into patient records had been delayed, and it had recorded 122 patient safety incidents involving incorrect, unavailable or delayed results.[5] **United States:** Ascension told patients it expected to rely on downtime procedures for some time.[3] Recovery has its own risks: results back-entered weeks later, duplicate records and patients lost to follow-up.

Blood is the sharpest edge. When blood cannot be matched in the usual way, transfusion falls back on O type blood, and stocks come under pressure. Every transfusion laboratory needs a downtime plan agreed with clinicians, including who authorises O type use.

Good downtime procedures are short, printed and practised. They name the forms, the phone numbers, the runners and the order of priority: emergency department, critical care, maternity, theatres, then the rest. They say how results will be checked and entered when systems return. And they are tested with the people who will use them, not just the people who wrote them.

Treat a cyber attack differently from a power cut in one way: you may not be able to trust the systems even when they come back. Keep digital and clinical leads in the same incident meeting, and decide your out-of-band channel now, before staff choose one for you.

DOWNTIME CHECKLIST

Downtime procedure checklist by department

Check each folder before the day and use it as the switch-over list on the day, in this order. It applies whether the cause is cyber, a supplier or the plant room.

DEPARTMENT	ON SWITCHING TO DOWNTIME	BEFORE SYSTEMS COME BACK
Emergency department	Paper triage and requests; urgent results phoned to a named number.	Every paper request matched to a result; discharges reviewed.
Critical care	Paper charts; blood gases and infusions logged by hand.	Charts reconciled and entered, checked by a second person.
Maternity	Paper notes; urgent bloods phoned; transfusion route confirmed.	Results confirmed in the record; babies linked to the right mother.
Theatres	Lists reviewed by clinical leads; cases needing blood held unless authorised.	Cancelled patients rebooked by clinical priority.
Laboratory and transfusion	Downtime plan started; manual reporting; O type use authorised by a named clinician.	Results back-entered and validated; duplicates resolved.
Pharmacy	Paper prescribing and dispensing; high-risk medicines double-checked.	Prescriptions and administration entered; allergies re-confirmed.
Imaging	Paper requests; urgent reports phoned; images held locally where safe.	Reports matched to requests and to the right patient.
Wards	Downtime folder opened; result phone numbers on display.	Paper records entered; results chased before discharge.

RESULTS LOG

Downtime results and back-entry log

One sheet per department, kept by the nurse in charge. Every result received on paper or by phone is written here, then entered and checked when systems return.

TIME	PATIENT (ID NUMBER)	TEST	RESULT RECEIVED FROM	GIVEN TO AND ACTED ON BY	ENTERED ON SYSTEM (DATE, INITIALS)	CHECKED BY
09:42	*TEST-000018*	*Potassium*	*Lab, ext. 4412, A.B.*	*Ward 7 doctor; repeat ordered*	*Day 3, C.D.*	*E.F.*

Illustrative example row; the identifier is not a real patient. Keep the log with the patient's paper notes. A result is only closed when it has been entered on the system and checked by a second person.

SYSTEM PARTNERS

Who needs to hear from you, and when

A declared incident is rarely yours alone. Agree these routes in advance and test them in exercises.

Regional health authority

Tell your health authority or commissioner when you declare, in the agreed format. **United Kingdom:** the integrated care board, which reports to NHS England through regional on-call.[6]

Ambulance service

If you need diverts or redirection, say so early, and tell them again when you can accept patients normally.

Suppliers and partners

Laboratory, imaging, patient record and facilities partners belong on your call-in lists, and you on theirs.

Primary care and the public

Family doctors, care homes, pharmacies and patients need to know what still works and where to go instead.

Cyber incidents also start regulatory clocks: see the regional notes in Part 01 for who reports what, and how fast.

LONG INCIDENTS

Heat, surge and the incident that does not end

Many healthcare incidents are rising tides, not explosions. They need a different rhythm.

PRESSURE	WHAT OFTEN HAPPENS	WHAT GOOD LOOKS LIKE
Seasonal surge	Escalation drifts to the highest level without a declared incident or extra staff.	Triggers that link your escalation levels to the severity ladder and call-in lists.
Heat	Chillers and server rooms fail together; theatres and scanners stop.	Temperature triggers, cooling checks and a plan for diagnostics, as one English hospital learned in June 2026.[4]
Long cyber recovery	Staff run on paper for weeks with no rota change or welfare plan.	A recovery cell, shift patterns, welfare checks and a plan for back-entering results.
Lone and community staff	Visits continue with no way to raise an alarm if systems are down.	Check-in times, one-touch alarms and visit lists printed before each shift.
Communication fatigue	Updates stop once the crisis feels normal.	Fixed update times for staff and patients until stand down.

YOUR PLAN

First week, first month, first quarter

Owners are suggestions. Adjust them to your structure.

WHEN	ACTIONS	OWNER
First week	Write declaration triggers and who can declare. Check call-in lists by role, with tested numbers for every on-call rota.	Executive lead for emergency preparedness
First week	Confirm the out-of-band channel if email and chat are lost. Print downtime forms for every clinical area.	Chief information officer
First month	Agree downtime plans with laboratory, transfusion, imaging and pharmacy partners, including results back-entry.	Chief medical officer
First month	Load the communication templates. Set a check-in rule for community and lone staff.	Head of comms; chief nurse
First month	Swap incident contacts with every clinical supplier, the ambulance service and your health authority.	Chief operating officer
First quarter	Link escalation levels, heat and cold alerts and generator and gas checks to the severity ladder.	Emergency management lead; estates
First quarter	Run a no-notice call-in test and a cyber downtime exercise with a key supplier. Time the first paper result.	Emergency management lead
First quarter	Report results, gaps and dated actions to the board, alongside your regulatory or accreditation assurance.	Executive lead for emergency preparedness

PRACTICAL TOOLKIT

Seven tools for your next incident

The same seven tools appear in every Crises Control playbook, with examples for your industry. Copy them, adapt them and rehearse with them.

01 **Readiness assessment** 28
Score eight areas from 0 to 3 and find your weakest link.

02 **Roles and responsibilities matrix** 29
Who is responsible, accountable, consulted and informed for each activity.

03 **Incident decision log** 30
Record every material decision, who made it and why, as it happens.

04 **Escalation workflow** 31
Four levels, their triggers, who is alerted and who decides.

05 **Communication templates** 32
Ready-to-send messages for staff, customers, regulators, partners and media.

06 **Tabletop exercise** 33
A ready-to-run scenario with timed injects and questions for the room.

07 **Post-incident review template** 34
Eight prompts that turn an incident into lessons and owned actions.

> Every tool works on paper. In Crises Control the same roles, escalation rules, templates and logs can be set up in the platform, so they run during the incident and leave a record afterwards.

TOOLKIT 01 · READINESS ASSESSMENT

How ready is your response?

Score each area from 0 (not in place) to 3 (fully in place) for your healthcare organisation, then add up the total.

AREA	THE QUESTION	SCORE 0 TO 3
01 · ACTIVATION **Alert**	Can you reach the right people on more than one channel within minutes, day or night? GOOD LOOKS LIKE Clinical, on-call and support staff reached on several channels, at any hour.	0 1 2 3
02 · ACTIVATION **Acknowledge**	Do you know who has seen the alert, and does silence trigger escalation by rule? GOOD LOOKS LIKE Call-in responses tracked, with escalation when key roles stay silent.	0 1 2 3
03 · COORDINATION **Escalate**	Are escalation paths set by role and time, not by whoever remembers who to call? GOOD LOOKS LIKE Declaration and escalation follow your emergency preparedness framework by role.	0 1 2 3
04 · COORDINATION **Coordinate**	Is there one shared workspace and log for the incident, not scattered calls, chats and inboxes? GOOD LOOKS LIKE One incident log shared by the incident team, clinical leads, partners and communications.	0 1 2 3
05 · EXECUTION **Execute**	Are your procedures turned into owned tasks with deadlines that someone can track? GOOD LOOKS LIKE Downtime procedures run as owned tasks in every affected department.	0 1 2 3
06 · EXECUTION **See**	Can leaders see tasks, people, locations and blockers live, without having to ask? GOOD LOOKS LIKE A live view of staff mobilised, services affected and actions outstanding.	0 1 2 3
07 · ASSURANCE **Prove**	Could you produce a timestamped timeline for a regulator, insurer or inquiry within a day? GOOD LOOKS LIKE A timeline that supports assurance, data protection regulators and the after-action review.	0 1 2 3
08 · ASSURANCE **Rehearse**	Do you exercise the whole response, not just the call tree, at least once a year? GOOD LOOKS LIKE Exercises that include a cyber attack on clinical systems, not just a major incident.	0 1 2 3

0 to 8
Alerting only
You can send messages. The response still depends on people and luck.

9 to 16
Coordinating in parts
Some steps work. Gaps appear at night, at scale or when systems fail.

17 to 24
Coordinated and provable
You can run the response and show exactly what happened.

TOOLKIT 02 · ROLES AND RESPONSIBILITIES MATRIX

Who does what in a critical incident

One accountable owner for every decision. Adapt the roles to your own on-call and incident command structure; clinical leads include laboratory, transfusion, emergency department, theatres and nursing.

ACTIVITY	EXECUTIVE ON CALL	INCIDENT COMMANDER	CLINICAL LEADS	DIGITAL AND IT	COMMS	HEALTH AUTHORITY AND PARTNERS
Grade the incident and call staff in	I	A	C	C	I	·
Declare a critical or major incident	A	R	C	C	I	I
Start downtime procedures in each department	I	A	R	C	·	·
Isolate systems and file cyber and data reports	I	C	I	A	·	I
Prioritise care: what is cancelled, diverted or protected	A	R	R	I	I	C
Authorise O type blood use under downtime	I	I	A	·	·	C
Request mutual aid and ambulance diverts	A	R	C	·	·	R
Messages to staff, patients and the public	A	C	C	I	R	I
Decide when systems are safe to restore	A	C	C	R	I	I
Back-enter and check results recorded on paper	I	A	R	R	·	·
Stand down and hold the hot debrief	A	R	C	C	C	I

R Responsible: does the work A Accountable: owns the decision C Consulted I Informed

> Illustrative. Check that every row has exactly one A, that the A can be reached at 03:00, and that a named deputy is on the call-in list for each role.

TOOLKIT 03 · INCIDENT DECISION LOG

Every decision, who made it and why

Kept by the incident loggist from the moment of declaration. Clinical decisions made under downtime, such as O type use, belong here as well as in the patient record.

TIME	DECISION	OPTIONS CONSIDERED	DECIDED BY	RATIONALE	EVIDENCE OR REFERENCE
08:52	*Authorise O type red cells for emergency use while crossmatch is unavailable*	*Wait for manual crossmatch; transfer the patient; O type use*	*Transfusion lead, with the emergency department consultant*	*Electronic crossmatch down; patient bleeding; blood bank confirmed stock*	*Log entry 14; blood bank stock check*

Log decisions as they are made, not afterwards. Include decisions not to act. The log is the backbone of the timeline your board, auditors and regulators will ask for.

TOOLKIT 04 · ESCALATION WORKFLOW

How an incident climbs, and who decides at each step

From the first report on a ward to a system-wide response. Times are illustrative: set your own and write them into the plan. Never wait at a level because the next one feels too big.

	TRIGGER	WHO IS ALERTED	WHO DECIDES
LEVEL 1 **Department response**	A system is slow or failing in one service, with a working fallback. Patient care is not yet affected.	Department lead, digital service desk, site manager.	Department or site manager. Logs the fault as a possible incident, not a ticket.

↓ ESCALATE IF NO FIX IN 30 MINUTES, CARE IS AFFECTED OR A CYBER CAUSE IS POSSIBLE

	TRIGGER	WHO IS ALERTED	WHO DECIDES
LEVEL 2 **Business continuity incident**	A clinical system or utility is down beyond the agreed time, or special arrangements are needed. SEV 3 on the ladder.	Manager on call, service leads, digital, estates, emergency management lead. Call-in by role, with acknowledgement.	Manager on call declares and starts downtime procedures.

↓ ESCALATE IF CRITICAL SERVICES ARE LOST, PEOPLE ARE AT RISK OR RANSOMWARE IS SUSPECTED

	TRIGGER	WHO IS ALERTED	WHO DECIDES
LEVEL 3 **Critical incident**	Loss of critical services or risk of harm; suspected cyber attack on a clinical system. SEV 2 or SEV 1.	Executive on call, incident commander, clinical leads, comms; health authority told at once.	Executive on call declares, in the agreed words, and opens the command centre.

↓ ESCALATE IF OTHER HOSPITALS ARE AFFECTED, CASUALTIES ARE ARRIVING OR HELP IS NEEDED FROM OUTSIDE

	TRIGGER	WHO IS ALERTED	WHO DECIDES
LEVEL 4 **System or major incident**	Several providers affected, mutual aid needed, ambulance diverts, or a major incident declared.	Regional health authority, ambulance service, suppliers, emergency services as needed.	Your executive still decides for your hospital. The health authority coordinates the system and mutual aid.

Notify your regional health authority when you declare. **United Kingdom:** notify the integrated care board, which reports to NHS England through regional on-call.[6] Step down as deliberately as you stepped up, with a message at each level.

TOOLKIT 05 · COMMUNICATION TEMPLATES

Six messages ready to send

Short, specific and honest. Fill the brackets, do not speculate on cause, and always give the time of the next update.

SMS · ON-CALL STAFF

137 chars

> [Hospital] critical incident: lab systems down. Incident team to [room] now. Reply 1 coming, 2 later, 3 not available. Next update 09:00.

Send by role. Escalate no reply after 10 minutes.

PUSH · ALL CLINICAL STAFF

> Laboratory systems unavailable. Use paper request forms and phone urgent results via [extension]. Transfusion is on downtime procedures. Do not use [system] until told. Update at 09:30.

Pin it in the app; repeat at each handover.

SMS · PATIENTS BOOKED TODAY

131 chars

> [Hospital]: an IT problem is affecting blood tests. Please attend as planned unless we contact you. Info: [web page]. Do not reply.

Approved by clinical leads first.

EMAIL · HEALTH AUTHORITY AND REGULATOR

> Critical incident declared by [hospital] at [time]: [system] unavailable, cause under investigation. Patient impact: [summary]. Reports filed: [ref]. Requests: [mutual aid]. Next update [time].

From the incident log, not from memory.

EMAIL · SUPPLIERS AND PARTNER LABORATORIES

> [Hospital] critical incident since [time]. Please send your incident lead to our call at [time] and route urgent samples via [route]. Contact: [name, number].

Agree the joint log and update times on the first call.

WEB AND MEDIA · PUBLIC HOLDING STATEMENT

> We have declared a critical incident because of an IT problem affecting laboratory services. Emergency care is open. For non-urgent needs, please use your usual doctor or pharmacy. We will update this page at [time].

Same words on the switchboard message.

TOOLKIT 06 · TABLETOP EXERCISE

Exercise: the lab goes dark

A discussion exercise built on Scenario 1 in this playbook. Read each inject aloud, give the room five minutes, and record decisions and gaps as you go. Run Scenarios 2 and 3 the same way.

DURATION	PARTICIPANTS	OBJECTIVE	FACILITATOR
Two and a half hours, plus a 30-minute hot debrief	**Executive and manager on call, laboratory, transfusion, emergency department, digital, comms, health authority liaison, supplier**	**Test declaration, call-in and the first paper result with no clinical systems**	**Emergency management lead, with an observer timing each decision**

00:00	INJECT 1	Monday 08:10. The lab manager reports the laboratory system is down. A ransom note is found on a server.	DISCUSS Who grades it, and how? Who is called first? What do wards do right now?
00:15	INJECT 2	Ten minutes after the call-in, the transfusion on-call and the theatre coordinator have not replied.	DISCUSS Who are the deputies? How do you know, now, who is coming in?
00:30	INJECT 3	Digital advise that hospital email and chat cannot be trusted.	DISCUSS What is the out-of-band channel? Which reports are due, to whom, and by when?
00:50	INJECT 4	The emergency department has a patient with major bleeding. Electronic crossmatch is unavailable.	DISCUSS Who authorises O type blood, how quickly, and where is it recorded?
01:15	INJECT 5	A local journalist calls. Patients are posting about cancelled appointments.	DISCUSS Who approves the holding line? What do today's patients hear, and from whom?
01:45	INJECT 6	The supplier says full restoration will take weeks.	DISCUSS Who runs recovery? What changes for the second shift, mutual aid and results back-entry?

> Measure three times: to grade the incident, to hear back from every key role, and to the first paper result. Put each gap on the actions board with an owner.

TOOLKIT 07 · POST-INCIDENT REVIEW TEMPLATE

Post-incident review template

Hot debrief within 48 hours of stand down, then a structured review. **United Kingdom:** the EPRR framework expects a structured debrief within 28 days.[6] **United States:** CMS expects real events and exercises to be analysed and documented.[9]

INCIDENT _____ DATE _____ SEVERITY _____ REVIEW LEAD _____

01 What happened, and when

The timeline from first report to stand down, taken from the incident log, not from memory.

02 Declaration and decisions

Were the triggers clear? Who declared, at what time, and was it early enough? Who decided what?

03 Call-in and acknowledgement

How long to reach, and hear back from, each key role? Who was not reached, and why?

04 Downtime procedures

Did each department switch to paper safely? How long to the first paper result?

05 Patient safety

Incidents reported, delays to care and results, harm reviews started, patients needing follow-up.

06 Partners and regulators

Did the health authority, ambulance service, suppliers and regulators hear in time, in the agreed words?

07 Staff, patients and the public

Were updates on time and consistent? Which channels worked? How were staff looked after?

08 Actions, owners, dates

What changes, who owns each one and by when. Add them to the actions board and the board report.

FOR LEADERS

Questions for your executive team

Ask them out loud. Vague answers are the finding.

READINESS

- ? Who can declare a critical incident tonight, and do they know the triggers?
- ? How long would it take to know who is coming in after a call-in?
- ? Could each ward get an urgent blood result with no computers?

PARTNERS AND PEOPLE

- ? Are our laboratory, imaging and patient record suppliers on our incident call lists, and are we on theirs?
- ? Which cyber and data reports would we owe, to whom, and by when?
- ? Who looks after the people who worked the first twelve hours?

CHECKLIST

Ready for minute one?

Tick what is true today. Anything blank is your next action.

- [ ] **Declaration triggers written and owned**
  Using your framework's definitions and standard formats.

- [ ] **Call-in lists by role, with acknowledgement**
  Tested out of hours in the last quarter.

- [ ] **Downtime procedures printed and practised**
  Including transfusion and results back-entry.

- [ ] **Out-of-band channel agreed**
  For use when email and chat cannot be trusted.

- [ ] **Suppliers on your incident lists**
  And your team on theirs.

- [ ] **Reporting routes and clocks known**
  Cyber, data protection and health regulator.

- [ ] **All three scenarios exercised**
  Cyber or IT downtime, mass casualty or surge, utility failure.

- [ ] **Hot debrief within 48 hours of stand down**
  Then a structured review with dated actions and welfare support.

GLOSSARY

Words worth knowing

Incident command
A standard structure of roles for running an incident, led by an incident commander.

HICS
Hospital Incident Command System: an incident command framework for hospitals, used widely in the US.

Emergency operations centre
The room or virtual space from which the incident is run; also called a command centre.

EPRR
Emergency preparedness, resilience and response: the NHS England framework for incidents (UK).

Business continuity incident
Disruption below acceptable levels that needs special arrangements until services recover.

Critical incident
Loss of ability to deliver critical services, or risk of harm to patients and staff.

Major incident
An event with serious consequences needing special arrangements by one or more responder agencies.

Mass casualty incident
More casualties than normal resources can manage, needing a declared, coordinated response.

METHANE
A format for sharing incident information: major incident, exact location, type, hazards, access, number of casualties, emergency services.

SBAR
Situation, background, assessment, recommendation: a structured way to escalate.

Downtime procedure
How a service works safely when a clinical system is unavailable.

Mutual aid
Help from other organisations, such as testing capacity or staff, during an incident.

Out-of-band channel
A way to communicate that does not depend on the systems that may be compromised.

Back-entry
Entering results and notes recorded on paper during downtime into the system once it is safe.

NIS2
The EU directive on cyber security risk management and incident reporting, which covers health.

Hot debrief
A short review straight after an incident, usually within 48 hours of stand down.

WHY COORDINATION, NOT JUST NOTIFICATION

Most organisations can send alerts. Few can manage incidents end-to-end.

When pathology goes down at 08:10, the alert is minute one. Keeping patients safe depends on who declared what, which staff came in, which downtime procedures started, what partners and patients were told, and a record for the after-action review.

WHERE INCIDENTS GO WRONG

01	**Fragmented information**	Different teams work from different information.
02	**Delayed decision making**	Leadership lacks visibility, so decisions wait.
03	**Uncoordinated response**	Several teams work independently on the same problem.
04	**Lack of visibility**	Critical actions become difficult to track.
05	**Lack of accountability**	Actions and decisions are poorly documented.

THE WHOLE INCIDENT, IN ONE PLATFORM



Crises Control is an **Operational Incident Coordination Platform**. It helps organisations maintain operational control during incidents by combining alerting, coordination, execution, visibility and accountability in one platform.

THE PLATFORM

One connected platform, from first alert to final report.

Most organisations can send alerts. Few can manage incidents end-to-end. Crises Control helps organisations maintain operational control during incidents by combining alerting, coordination, execution, visibility and accountability in one platform. Each module of this incident management software does one job; the value comes from how they work together across the incident.

01

ESCALATE

SOS

Frontline staff raise an alert from the mobile app, with location where available.

02

START

Ping

Alerting and activation: reach people and collect acknowledgements and responses.

03

COORDINATE

Incident Manager

Authorised users activate the incident and its response plan, assign roles, control communication and keep the timeline.

04

EXECUTE

Task Manager

Assign actions, track completion and escalate what stalls, with clear accountability.

05

OPERATE

Control Centre

Real-time view of incidents, timelines, task status and assets.

06

PROVE

Audit & Reporting

Incident records, response analysis and task accountability, ready to report.

07

ASSIST

CRAiG

AI assistant (beta) that answers questions and summarises incident data. People make every decision.

THE OUTCOMES

Alerting and activation

Reach the right people immediately.

Operational coordination

Structure communication and response activities.

Execution and visibility

Track tasks, responsibilities and progress.

Proof and accountability

Create a complete, defensible operational record.

Most organisations start with the capability they need today, often alerting, and expand as their requirements mature. Highlighted modules are the ones this playbook draws on.

HOW THE MODULES WORK TOGETHER

How the modules work together in a hospital incident

Crises Control is an Operational Incident Coordination Platform. Here one laboratory outage is followed through the platform. It supports your emergency plan, incident command structure and business continuity plans; it does not replace them.

01 ESCALATE
SOS

A laboratory manager, or a lone community nurse, raises an alert in one touch from the app, with location where available, so the site team knows in seconds.

02 START
Ping

On-call, clinical and digital leads are called in by role by push, SMS, voice and email; replies show who is coming, and no reply escalates to a deputy under the rules you configure.

03 COORDINATE
Incident Manager

An authorised user declares the incident and launches the response plan; roles are assigned, and the incident team, partner liaison and supplier work from one communication log.

04 EXECUTE
Task Manager

Downtime procedures, O type authorisation and results back-entry run as tasks with owners and deadlines, assigned when the plan is activated.

05 OPERATE
Control Centre

The incident team sees who was notified and acknowledged, task progress and locations, without chasing wards for updates.

06 PROVE
Audit & Reporting

A timestamped record of messages, acknowledgements, decisions logged and tasks, exportable as PDF or CSV, supports the debrief, regulatory reports and accreditation evidence.

07 ASSIST
CRAiG

CRAiG (beta) answers questions and summarises the incident's data in the platform, within each user's permissions. People make every decision.

COORDINATION
Everyone works from the same incident, roles and log.

VISIBILITY
Leaders see progress without chasing for updates.

EXECUTION
Every action has an owner, a deadline and an escalation.

ACCOUNTABILITY
The record of what happened builds itself as you go.

WHAT IS STANDARD, WHAT IS CONFIGURED

Clear about what the platform does.

Some capabilities work as standard. Others depend on how your team sets up plans, groups and rules, or on connecting your own systems. Your demonstration and implementation plan confirm what applies to you.

STANDARD

Included in how each module works.

- SOS press-and-hold alert from the mobile app, raising an incident and sharing location where available
- Ping alerts by push, SMS, voice, email and desktop, with one-tap acknowledgement
- Incident Manager workspace, roles and a single communication log
- Task Manager tasks with owners and deadlines when a plan is activated
- Control Centre live view of status, acknowledgements, tasks and locations
- Audit & Reporting: timestamped records, exportable as PDF or CSV
- CRAiG (beta): answers questions and summarises your incident data within each user's permissions

CONFIGURED

Set up by your team to match your plans.

- Response plans, task lists and message templates per incident type
- Groups, sites and audiences to target
- Escalation rules and timings when people do not respond
- Cascading from one channel to the next
- Response options people can choose when they acknowledge

INTEGRATIONS

Connected to your other systems, where configured.

- Directory sync with Microsoft Entra ID, SCIM or scheduled file import
- Single sign-on with Entra ID, Google Workspace or Okta
- The Microsoft Teams app, enabled by your administrator
- Triggers from monitoring or other systems, set up as an integration project

> Crises Control does not detect incidents on its own. Incidents are raised by people, through SOS, the app or the web, or by connected systems where an integration has been configured. From that point the platform coordinates the response.

USE CASES

How organisations use Crises Control in healthcare

Code Blue and urgent clinical team mobilisation

CUSTOMER EXAMPLE

TRIGGER A medical emergency requires an immediate response from a designated clinical team

Allow authorised staff to launch the incident, alert the response team, track individual acknowledgements and monitor staff availability.

Incident Manager | Ping | acknowledgement tracking | Control Centre

Code White and Code Yellow response

CUSTOMER EXAMPLE

TRIGGER A hospital activates an established internal emergency protocol

Notify relevant management and response personnel, assign protocol-related tasks and coordinate the response in line with hospital procedures.

Incident Manager | Ping | Task Manager | Control Centre | Audit & Reporting

Major incident and multi-department coordination

SUGGESTED APPLICATION

TRIGGER A major event requires clinical, operational, digital and management teams to work together

Activate the relevant response group, coordinate communications, distribute actions and monitor progress across teams.

Ping | Incident Manager | Task Manager | Control Centre | Audit & Reporting

Disaster recovery and system downtime

CUSTOMER EXAMPLE

TRIGGER A hospital information system or other critical technology service is disrupted

Communicate recovery or downtime procedures, distribute tasks across departments and coordinate the response. Automated plan triggering depends on configured integrations and workflows.

Ping | Incident Manager | Task Manager | integrations where configured | Audit & Reporting

Temperature monitoring and cold-storage alerts

CUSTOMER EXAMPLE

TRIGGER A warehouse cooling system moves outside its acceptable temperature range

Use configured monitoring triggers to alert relevant employees when temperatures fall outside defined thresholds, then coordinate follow-up actions.

Ping | configured integrations or triggers | Task Manager

IT and operational disruption notifications

CUSTOMER EXAMPLE

TRIGGER An IT service or business application becomes unavailable

Notify relevant IT teams and employees, provide updates and coordinate any required follow-up actions.

Ping | Incident Manager | Task Manager | Audit & Reporting

On-call and surgical team mobilisation

CUSTOMER EXAMPLE

TRIGGER An urgent clinical situation requires rapid mobilisation of on-call physicians or surgical teams

Alert the designated team, collect acknowledgements and availability, and provide visibility of who has responded.

Ping | Incident Manager | acknowledgement tracking | Control Centre

Simulation tests and response readiness

CUSTOMER EXAMPLE

TRIGGER Clinical teams need to practise emergency procedures and confirm readiness

Run simulation incidents and test alerts, verify that teams can be reached and review participation and response records.

Ping | Incident Manager | Task Manager | Audit & Reporting

Customer examples are based on how customers use Crises Control today; customer names have been removed. Customer requirements describe needs raised by customers or in evaluations. Intended, suggested and illustrative use cases show how the platform can be applied and are not presented as existing customer deployments. Capabilities marked 'where configured' or 'where supported' depend on each organisation's set-up.

BUYER'S GUIDE

Eight questions to ask any incident platform supplier

Many tools can send an alert. These questions separate a notification tool from a platform that coordinates the whole response.

01 **Does it stop at the message?**
Ask what the platform does after the alert is sent: tasks, log, escalation, recovery.

02 **Can it prove who acknowledged?**
Delivery is not acknowledgement. Ask how silence is spotted and escalated, and what you must configure.

03 **Does it run your procedures?**
Ask whether your plans become owned tasks with deadlines, or stay as documents.

04 **Is there one shared picture?**
Ask what leaders see live, and whether it needs someone to update a spreadsheet.

05 **Does it work when your systems do not?**
Ask how teams coordinate if email, chat or your network is down.

06 **Can frontline staff raise the alarm?**
Ask how a colleague on site escalates, with their location where available, in one action.

07 **What evidence does it leave behind?**
Ask for a sample timeline of the kind a regulator, insurer or inquiry would want.

08 **Can you rehearse in it?**
Ask whether you can run exercises on the same platform you would use for real.

SOURCES

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BOOK YOUR HEALTHCARE PLATFORM DEMONSTRATION

Watch a pathology outage run end to end.

Most organisations can send alerts. Few can manage incidents end-to-end. In a live, 30-minute demonstration built around a healthcare incident, you will see how our incident management software, an Operational Incident Coordination Platform, takes one incident from first alert to final report. Bring your own scenario.

WHAT YOU WILL SEE

01	SOS	A laboratory manager reports a system outage, with location where available.
02	PING	On-call, clinical and digital leads are alerted; call-in responses come back.
03	INCIDENT MANAGER	The incident is declared, roles assigned and partners added.
04	TASK MANAGER	Downtime procedures in each department become owned tasks.
05	CONTROL CENTRE	The incident team sees services affected, staff mobilised and actions outstanding.
06	AUDIT & REPORTING	The record for assurance, regulators and the after-action review is produced.
07	CRAIG	CRAiG helps the incident team find information in the platform and summarise the incident so far.

Book your demonstration →

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SCAN TO BOOK

COMPANION REPORT When the lab went dark